

Adult Type 2 Diabetes Management Algorithm

Adaptation of 2026 American Diabetes Association Standards of Care in Diabetes – 2026 &
2026 American Association of Clinical Endocrinology Consensus Statement: Algorithm for Management of Adults with Type 2 Diabetes – 2026 Update

Overview of Type 2 Diabetes Care

Diagnosis

- A1c $\geq 6.5\%$
- FPG ≥ 126 mg/dL
- 2h 75g OGTT ≥ 200 mg/dL
- Random glucose ≥ 200 mg/dL + symptoms

Note: Confirm with repeat test in the absence of classic symptoms

Start with monotherapy unless

- A1c $\geq 1.5\%$ above glycemic target $\rightarrow$ consider dual therapy
- A1c $\geq 10\%$ + symptoms, consider combination injectable therapy w/ basal insulin or GLP-1 RA

Standards of Diabetes Care

- A1C q3m (uncontrolled); q6m (well controlled)
- Statin for > 40 y/o; lipids annually
- UACR & BMP annually $\rightarrow$ ACEi or ARB w/ UACR > 30
- Annual comprehensive foot exam
- Annual dilated eye exam
- Dental referral for comprehensive exam
- Aspirin per ADA/USPSTF guidelines
- Recommended Vaccines
 - RSV: ≥ 60 y/o (single dose)
 - Hep B: < 60 y/o (3 doses)
 - Zoster: ≥ 50 y/o (2 doses)
 - Tdap: All adults (booster every 10 years)
 - PPSV23: 19-64 y/o + ≥ 65 y/o (1 dose if received PCV13)
 - PCV20 or PCV15: 19-64 y/o & immunocompromised (1 dose)
 - Influenza ≥ 6 months (annually) - avoid live vaccine)
 - COVID-19: ≥ 6 months (initial vaccines + boosters)

Treatment Goals

- A1c $< 6\%$: Adults considering pregnancy
 - A1c $< 7\%$ (FBG 80 – 130 mg/dL): “Healthy,” non-pregnant adults
 - TIR 70 – 180 mg/dL $> 70\%$
 - TBR < 70 mg/dL $< 4\%$
 - A1c $< 8\%$ (FBG 90 – 150 mg/dL): $\uparrow$ risk of hypoglycemia, limited life expectancy, complex health status, or long-standing DM that is difficult to control
 - TIR 70 -180 mg/dL $> 50\%$
 - TBR < 70 mg/dL $< 1\%$
 - BP Goal $< 130/80$ mmHg for most
 - SBP goal < 120 mmHg – High CVD/Kidney risk
 - BP goal $< 140/90$ mmHg – poor health, limited life expectancy, high risk for adverse effects of therapy
- Note:** A1c goals should be patient-centered and reflect what is safe/easily attainable.

Commented [PA1]: Try to include box on technologies available (CGM, smart pens, AID systems, etc.)

Commented [PA2R1]: Include recommendation for DSMES referral in Standards of Care section.

Commented [PA3]: Update wording to emphasize need to screen for CKD (UACR and GFR) and perhaps elaborate on treatment options

Commented [LT4R3]: High CVD and Kidney risk - systolic blood pressure goal has been updated to < 120 mmHg

Commented [LT5R3]: Use of dual GIP and GLP-1 RAs - for patients with type 2 diabetes and symptomatic heart failure (HFrEF) or MASH

Commented [LT6R3]: Consider SGLT2 inhibitor + nsMRA initiation for pts with albumin-to-creatinine ratio ≥ 100 mg/g with eGFR 30–90 mL/1.73 m² on a renin- angiotensin-aldosterone system (RAS) inhibitor

Commented [PA7]: Comment on Gluco-centric management vs complications/risk-based management

Individual T2DM Medication Characteristics

	Incretin Mimetics*	SGLT-2 Inhibitors	DPP-4 Inhibitors*	Thiazolidinediones (TZDs)	Sulfonylureas
Medications	GLP-1 Receptor Agonists Exenatide Bydureon BCise (exenatide ER) Ozempic [SQ]/Rybelsus [PO] (semaglutide) Trulicity (dulaglutide) Victoza (liraglutide) Dual GLP-1/GIP Agonist Mounjaro (tirzepatide)	Brenzavvy (bexagliflozin) Farxiga (dapagliflozin) Invokana (canagliflozin) Jardiance (empagliflozin) Steglatro (ertugliflozin)	Alogliptin Januvia (sitagliptin) Onglyza (saxagliptin) Tradjenta (linagliptin)	Pioglitazone Rosiglitazone	Glipizide Glimepiride Glyburide (AVOID USE)
Potential A1c $\downarrow$	1.1 – $> 2\%$	0.5 – 1%	0.4 – 0.8%	0.5 – 1.5%	1 – 2%
Hypoglycemia	Low risk	Low risk	Low risk	Low risk	High risk
Weight	Loss	Loss	Neutral	Gain	Gain
Side effects	GI (N/V/D, $\downarrow$ appetite) BBW: Risk of thyroid tumors in animal studies (Avoid if Hx of MTC or MEN2)	GU infections, dehydration/fluid loss	Neutral/flu-like symptoms	Edema, HF exacerbation, bone fractures	Hypoglycemia, weight gain
CVD Risk Reduction/ CV Harm	MACE Dulaglutide Liraglutide Semaglutide (SQ + PO)	MACE Canagliflozin Empagliflozin	MACE Neutral	MACE Potential benefit: pioglitazone	MACE Neutral
$\downarrow$ Progression of DKD	Dulaglutide Liraglutide Semaglutide (SQ)	Empagliflozin Canagliflozin Dapagliflozin	Neutral	Neutral	Neutral

Commented [PA8]: May include brand name of meds that are missing brand names

Commented [PA9R8]: Will not include to save space.



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MASH	Semaglutide (SQ) Tirzepatide (potential)	Unknown	Unknown	Potential benefit	Unknown
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ACEi: angiotensin-converting enzyme inhibitor; ARB: angiotensin II receptor blocker; ASCVD: atherosclerotic cardiovascular disease; BBW: black box warning; BMP: basic metabolic panel; BP: blood pressure; CVD: cardiovascular disease; DKD: diabetic kidney disease; DPP-4: dipeptidyl peptidase 4; DSMES: diabetes self-management education & support; FBG: fasting blood glucose; FPG: fasting plasma glucose; GI: gastrointestinal; GIP: Glucose-dependent Insulinotropic Polypeptide; GLP-1 RA: glucagon-like peptide-1 receptor agonist; GU: genitourinary; HF: heart failure; Hx: history; **MACE**: Major adverse CV events; **MASH**: metabolic dysfunction-associated steatohepatitis; MEN2: Multiple Endocrine Neoplasia syndrome type 2; MTC: medullary thyroid carcinoma; nsMRA: nonsteroidal mineralocorticoid receptor antagonist; N/V/D: nausea, vomiting, diarrhea; OGTT: oral glucose tolerance test; PG: plasma glucose; PO: oral; SGLT-2i: sodium-glucose cotransporter 2 inhibitor; T2DM: type 2 diabetes mellitus; TBR: time below range; TIR: time in range; UACR: urine albumin-to-creatinine ratio

*Concomitant use of incretin mimetics and DPP-4 inhibitors not recommended due to potential risk of pancreatitis and no additional A1c reduction.

Pharmacologic Approaches to Managing Diabetes

1. First-Line Therapy

If not at goal at 3 months of monotherapy, continue to dual therapy

(1 -2% A1c ↓)
GOAL DOSE: 1000 mg BID
Contraindicated if eGFR < 30
Caution w/ eGFR 30-45
Switch to ER formulation to ↓ GI side effects

Consider patient characteristics to help guide decision making for the next agent used.

Lifestyle Modifications

DSMES + MNT Referral

Smoking Cessation
Diet/Physical Activity/Weight Loss
(> 5% - 7% weight loss)

Investigate social determinants of health and refer to appropriate team member

Commented [PA10]: Add MASH abbreviation

Commented [PA11R10]: Add "MACE" abbreviation

Commented [PA12]: Include nsMRA in the algorithm (if not, remove abbreviation)

Commented [PA13]: Will add "potential benefit on MACE"

Commented [PA14]: Comment on not needing to wait 3 months to make therapy changes to avoid therapeutic inertia

Commented [PA15]: Emphasize lifestyle modifications more (move this box first) and add "referral to self-management education (DSMES)"

2. Indicators of High-Risk or Established ASCVD, CKD, or HF?

NO

Consider these other patient scenarios:

ASCVD/High CVD Risk

GLP1-RA w/
CV benefit
OR
SGLT-2i w/
CV benefit

HF

HFrEF/HFpEF
SGLT-2i w/
proven benefit
OR
SGLT-2i and/or
dual GIP/GLP-1
RA or GLP-1 RA
w/ proven
benefit

CKD (on max tolerated ACEi/ARB)

SGLT-2i w/ evidence of
reducing CKD
progression
• Can be started w/
eGFR ≥ 20
• Glucose lowering
efficacy ↓ w/ eGFR
<45
OR
GLP1-RA w/ CKD
benefit

Minimize Hypoglycemia

• GLP-1 RA
• SGLT-2i
• DPP-4i
• TZD

Weight Management

• GLP-1 RA
○ ~3-14 lbs.
○ Semaglutide>
liraglutide>
exenatide>
dulaglutide
• SGLT-2i
○ ~4-7 lbs.

Cost Issues

• TZD
• Sulfonylurea
OR
• Unity's 340B
pharmacy
resources
• Prescription
med coupons

If not at goal after maximizing dose of dual therapy continue to triple therapy

3. Triple Therapy

If not at goal after maximizing dose of triple therapy proceed to quadruple therapy OR combination injectable

Metformin

+

GLP-1 RA
SGLT2 Inhibitor
DPP-4 Inhibitor (not
used with GLP-1 RA)
TZD
Sulfonylurea

+

GLP-1 RA
SGLT2 Inhibitor
DPP-4 Inhibitor (not
used with GLP-1 RA)
TZD
Sulfonylurea

4. Combination Injectable Therapy

Oral Agents

+

GLP-1 RA

+

Basal Insulin
Titrate to FBG goal

Consider GLP-1 RA in MOST patients prior to insulin.
Consider insulin as first injectable if:

- A1c very high (>10%)
- Symptoms or evidence of catabolism: weight loss,

Basal Insulin Considerations

Start: 0.2 – 0.3 Units/kg/day for most patients (monitor BG closely in patients with renal disease)



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Commented [PA16]: Ensure Chronic Care Diabetes Teams folder has the appropriate "additional resources"